HEALTH BEACON

CMSRN EXAM PREP

Practice Exam 01 - Patient/Care Management

40 scored practice questions | Printable Practice Exam

Instructions	Select the single best answer for each question. An answer key with explanations begins after the question section.

PART A - QUESTIONS

1.

Scenario

A 78-year-old patient with Parkinson disease was admitted for heart failure. After receiving a morning loop diuretic, the patient becomes lightheaded when standing and repeatedly tries to walk unassisted to the bathroom. The patient is alert, uses a walker at home, and has urgency.

Which addition to the plan of care best addresses the patient's immediate fall risk while preserving mobility?

A) Keep the patient on bed rest until the diuretic is discontinued

B) Schedule assisted toileting, place a bedside commode within reach, and supervise transfers

C) Apply a vest restraint whenever the patient is alone

D) Ask the family to remain at the bedside continuously

2.

Scenario

An immunocompromised adult is admitted with fever and a widespread vesicular rash. Disseminated herpes zoster is suspected.

Which room placement and personal protective equipment are appropriate while the diagnosis is evaluated?

A) A private room with gloves only

B) A private room with gown and gloves only

C) A positive-pressure room with a surgical mask

D) An airborne infection isolation room with a fit-tested respirator, gown, and gloves

3.

Scenario

A patient taking sertraline begins linezolid for a resistant infection. Several hours later, the patient develops agitation, diaphoresis, tremor, hyperreflexia, and inducible ankle clonus.

What should the nurse do next?

A) Hold the linezolid and promptly notify the prescriber of suspected serotonin toxicity

B) Give the next sertraline dose early to reduce medication withdrawal

C) Administer an antipyretic and reassess the temperature in 1 hour

D) Slow the linezolid infusion and continue both medications

4.

Scenario

Thirty minutes after receiving IV hydromorphone for postoperative pain, an opioid-naive patient is difficult to arouse, has a respiratory rate of 7/min, and has an oxygen saturation of 88% on room air.

Which response is most appropriate?

A) Allow the patient to sleep and repeat vital signs in 1 hour

B) Apply oxygen and give the next opioid dose by the oral route

C) Stop opioid administration, support ventilation, and give naloxone per protocol

D) Place the patient in Trendelenburg position and encourage oral fluids

5.

Scenario

A patient with chronic low-back pain and diabetic peripheral neuropathy requests a non-drug method that can be used during painful episodes. The patient has reduced temperature sensation over the lower back.

Which intervention is safest for the nurse to recommend?

A) Apply a heating pad on high until the pain subsides

B) Practice paced breathing and guided imagery during pain flares

C) Place an ice pack directly on the skin for 30 minutes

D) Use vigorous massage over areas with decreased sensation

6.

Scenario

A patient scheduled for a bedside pleural procedure received IV midazolam before signing the consent form. The patient is now drowsy and cannot explain the planned procedure. The proceduralist asks the nurse to witness the signature.

How should the nurse respond?

A) Decline to witness and notify the proceduralist that valid consent must be obtained when the patient has decision-making capacity

B) Witness the signature because the proceduralist is responsible for informed consent

C) Ask a family member to sign even though the patient has no designated surrogate

D) Sign the form for the patient because the procedure is already scheduled

7.

Scenario

A patient admitted with an acute stroke coughs after a sip of water and develops a wet, gurgling voice. No swallowing evaluation has been completed.

Which action is indicated?

A) Offer thickened liquids and observe the next swallow

B) Place pills in applesauce and continue oral medications

C) Encourage the patient to tuck the chin and retry water

D) Keep the patient NPO and request a swallowing evaluation

8.

Scenario

A patient with diabetes has a premeal glucose of 86 mg/dL and is prescribed rapid-acting insulin with breakfast. Dietary services reports that the tray will be delayed at least 45 minutes.

What is the safest nursing action?

A) Delay the prandial insulin until the meal is available, following the medication order and facility timing policy

B) Give the full dose now to maintain the medication schedule

C) Give the insulin now and provide only sugar-free gelatin

D) Omit all insulin for the day without contacting the prescriber

9.

Scenario

Four hours after thyroid surgery, a patient becomes restless and repeatedly swallows. The nurse observes increasing neck fullness and a muffled voice.

Which action should the nurse take?

A) Offer ice chips and reassess swallowing in 30 minutes

B) Lower the head of the bed to improve cerebral perfusion

C) Remove the surgical dressing to inspect the incision

D) Activate an urgent response for possible neck hematoma and prepare to support the airway

10.

Scenario

Before starting a blood transfusion, a nurse notices that the blood tubing was almost connected to the wrong patient's IV line. The error is corrected before blood reaches either patient, and neither patient is harmed.

What is the appropriate follow-up?

A) Document the other nurse's name in both patients' medical records

B) Take no action because the error did not reach a patient

C) Report the near miss through the safety reporting system and participate in review of contributing system factors

D) Wait to report the event unless the same error happens again

11.

Scenario

A patient newly prescribed warfarin says, “I eat kale several times a week, so I guess I have to stop eating it completely.”

Which teaching by the nurse is accurate?

A) “Avoid every food that contains vitamin K while taking warfarin.”

B) “Keep your vitamin K intake reasonably consistent and report major diet changes.”

C) “Eat extra kale whenever you miss a warfarin dose.”

D) “Vitamin K affects warfarin only if it is taken as a supplement.”

12.

Scenario

A patient's central venous catheter dressing is damp and lifting at one edge after a shower. The insertion site is not visible through the wet dressing.

What should the nurse do?

A) Reinforce all four edges with tape and change it on the scheduled date

B) Replace the dressing promptly using aseptic technique and the approved skin antiseptic

C) Remove the dressing and leave the site open to air until it dries

D) Apply antibiotic ointment under the existing dressing

13.

Scenario

A postoperative patient is using patient-controlled analgesia (PCA). The nurse enters the room and sees the patient's partner press the PCA button while the patient is asleep.

Which response by the nurse is appropriate?

A) Assess the patient's sedation and breathing, then explain that only the patient may press the PCA button

B) Ask the partner to press the button only when the patient appears uncomfortable

C) Disable the lockout interval so the patient will need less assistance

D) Remove the PCA immediately without assessing the patient

14.

Scenario

A patient with chronic obstructive pulmonary disease becomes short of breath while walking to the bathroom. The oxygen saturation remains within the prescribed range, and the patient is anxious but able to follow directions.

Which non-pharmacological technique should the nurse coach the patient to use?

A) Lie flat and take rapid, shallow breaths

B) Hold the breath after each inspiration for 15 seconds

C) Lean slightly forward and use slow pursed-lip exhalation

D) Breathe repeatedly into a paper bag

15.

Scenario

Two patients on the unit have similar names. A unit of packed red blood cells arrives for one of them. The intended recipient is awake and wearing a legible identification band.

Which identification process is safest before beginning the transfusion?

A) Ask the roommate to confirm which patient is receiving blood

B) Compare the room number with the label on the blood product

C) Use the patient's surname and diagnosis as the two identifiers

D) Match two approved patient identifiers against the order and blood product with the required independent verification

16.

Scenario

During moderate sedation for a bedside procedure, the patient begins snoring, responds only to repeated stimulation, and has an oxygen saturation of 89% despite supplemental oxygen.

What should the monitoring nurse do?

A) Increase the sedative to prevent sudden movement

B) Stop further sedation, open the airway, and call for immediate assistance

C) Continue the procedure because snoring is expected during moderate sedation

D) Wait for the oxygen saturation to fall below 85% before intervening

17.

Scenario

An older adult with a long-term urinary catheter has cloudy urine and a positive urine culture. The patient is afebrile, has no suprapubic or flank pain, and is at cognitive baseline. An antibiotic is prescribed solely for the culture result.

Which nursing action best supports antimicrobial stewardship?

A) Administer the antibiotic and obtain another culture after the first dose

B) Irrigate the catheter with antiseptic solution before giving the antibiotic

C) Clarify the clinical indication with the prescriber before starting treatment

D) Replace the antibiotic with a broad-spectrum agent from unit stock

18.

Scenario

During a peripheral IV infusion of vancomycin, the patient reports burning. The nurse observes swelling and coolness around the site, and the infusion pump is not alarming.

What should the nurse do first?

A) Increase the infusion rate to complete the dose sooner

B) Apply pressure over the site while the infusion continues

C) Stop the infusion and manage the suspected infiltration according to the medication-specific protocol

D) Silence the pump and reassess the site after the dose is complete

19.

Scenario

A patient with chronic osteoarthritis says, “If the treatment works, I should never feel any pain.” The patient avoids walking because movement may increase discomfort.

Which response best supports realistic pain-management goals?

A) “Complete pain relief is expected if you take opioids on schedule.”

B) “Pain ratings are more important than what activities you can perform.”

C) “You should avoid activity until your pain score stays at zero.”

D) “We can combine strategies to reduce pain enough to improve sleep and function.”

20.

Scenario

A hospitalized patient who recently lost a spouse says quietly, “There is nothing left for me, and everyone would be better off without me.” The patient then looks away and asks the nurse to leave.

Which response is indicated?

A) Reassure the patient that grief usually improves with time

B) Stay with the patient and directly assess suicidal thoughts, plan, means, and immediate safety

C) Avoid asking about suicide because it may increase the patient's distress

D) Document the statement and ask the next shift to reassess

21.

Scenario

A patient receiving continuous gastric tube feeding is found supine with the head of the bed at 10 degrees. The patient has no medical restriction on positioning.

Which intervention best reduces aspiration risk?

A) Elevate the head of the bed to at least 30 degrees during feeding

B) Turn the feeding off for 8 hours every night

C) Add blue dye to the formula to detect aspiration

D) Check gastric residual volume every 15 minutes

22.

Scenario

At admission, an older adult presents two medication lists that conflict. The patient uses a weekly pill organizer but cannot identify the tablets, and a recent hospital discharge added several prescriptions.

Which action best supports accurate medication reconciliation?

A) Verify the regimen using medication containers, pharmacy records, and recent discharge information before finalizing the list

B) Use the longest list because it is likely the most complete

C) Record only medications the patient can name from memory

D) Ask the patient to resume every listed medication until the next clinic visit

23.

Scenario

A young adult arrives with an older companion who insists on answering every question, holds the patient's identification, and refuses to leave. The patient avoids eye contact and has injuries in different stages of healing.

What is the nurse's best next step?

A) Arrange a private, trauma-informed interview using a qualified interpreter if needed and follow the facility safety protocol

B) Confront the companion in the hallway and accuse the person of trafficking

C) Ask the companion to translate sensitive questions for the patient

D) Give the patient a hotline card in front of the companion and discharge the patient

24.

Scenario

A patient with heart failure is learning to compare packaged foods for a prescribed sodium restriction. One package contains two servings.

Which statement shows correct label-reading technique?

A) “I will choose the product with the fewest calories, regardless of sodium.”

B) “The sodium percentage does not matter if the food is labeled natural.”

C) “I only need to count sodium added from the saltshaker.”

D) “I will multiply sodium per serving by the number of servings I actually eat.”

25.

Scenario

Two hours after a colonoscopy with polypectomy, a patient reports rapidly worsening abdominal pain. The abdomen is rigid and distended; heart rate is 118/min.

Which action is appropriate?

A) Encourage ambulation to relieve retained gas

B) Keep the patient NPO and obtain urgent medical evaluation for possible perforation

C) Apply a heating pad and offer oral fluids

D) Explain that severe pain is expected after polypectomy

26.

Scenario

A nurse attempts to flush an implanted port before administering medication. There is no blood return, and firm resistance is felt. The patient denies pain, and no swelling is visible.

What should the nurse do?

A) Use a smaller syringe to create more pressure

B) Continue pushing until the resistance is overcome

C) Stop, assess for mechanical causes, and follow the facility's occlusion-management protocol

D) Administer the medication because there is no swelling

27.

Scenario

Several patients on a medical unit develop sudden vomiting and watery diarrhea, and norovirus is suspected. A nurse is planning care for an affected patient.

Which practice is appropriate during the outbreak?

A) Use standard precautions only after symptoms improve

B) Use contact precautions and wash hands with soap and water after care

C) Share electronic equipment between rooms without disinfection

D) Wear an N95 respirator instead of a gown and gloves

28.

Scenario

A hospital plans to introduce new smart infusion pumps. Before implementation, a multidisciplinary team maps how programming errors could occur, estimates the likelihood and severity of each failure, and designs safeguards.

Which safety method is the team using?

A) Failure mode and effects analysis

B) Root cause analysis

C) Peer review after an adverse event

D) Retrospective chart audit

29.

Scenario

A patient receiving a continuous thoracic epidural infusion after abdominal surgery develops new bilateral leg weakness, blood pressure of 82/48 mm Hg, and increasing drowsiness.

Which action should the nurse take?

A) Assist the patient to walk to determine whether the weakness improves

B) Increase the epidural rate because pain may be causing hypotension

C) Place a heating pad over the catheter site

D) Stop the infusion, support circulation and airway as needed, and notify the anesthesia team immediately

30.

Scenario

A patient prescribed apixaban says, “I skip doses near the end of the month because I cannot afford the refill.” The patient has no current bleeding or thromboembolic symptoms.

What is the nurse's best response?

A) Suggest taking half-doses so the supply lasts longer

B) Advise doubling the next dose after each skipped dose

C) Recommend replacing apixaban with daily aspirin without an order

D) Notify the prescriber and connect the patient with medication-assistance resources before discharge

31.

Scenario

On postoperative day 2 after abdominal surgery, a patient coughs and the incision opens with bowel visible at the wound.

Which action should the nurse take?

A) Position the patient with knees flexed, cover the area with sterile saline-moistened dressings, and notify the surgeon urgently

B) Gently push the bowel back into the abdomen and apply a binder

C) Ask the patient to stand while the wound is inspected

D) Cover the wound with dry gauze and offer oral fluids

32.

Scenario

A confused patient repeatedly reaches for a nasogastric tube. The current plan uses wrist restraints, but the patient becomes more agitated and has not attempted to leave the bed.

Which action best reflects safe restraint management?

A) Tighten the restraints so the patient cannot reach the tube

B) Continue restraints indefinitely because a tube is present

C) Assess causes of agitation and use the least restrictive effective alternatives with frequent reassessment

D) Ask an unlicensed assistive person to renew the restraint order

33.

Scenario

A patient with suspected pulmonary tuberculosis must leave an airborne infection isolation room for an urgent CT scan.

Which preparation is appropriate for transport?

A) Place a surgical mask on the patient, limit transport to what is necessary, and notify the receiving department

B) Place an N95 respirator on the patient and remove it during hallway transport

C) Have the patient wear no mask if transport time is under 15 minutes

D) Discontinue airborne precautions once the patient leaves the room

34.

Scenario

On the first day after upper abdominal surgery, a patient takes shallow breaths and avoids using the incentive spirometer because the incision hurts with deep inspiration.

Which nursing intervention is most appropriate?

A) Tell the patient to avoid coughing until the incision heals

B) Help the patient splint the incision and coordinate deep breathing with the pain-management plan

C) Keep the patient supine while using the incentive spirometer

D) Replace deep-breathing exercises with continuous bed rest

35.

Scenario

A patient with diabetes and gastroparesis reports early satiety, bloating, and wide glucose swings after eating two large meals daily.

Which nutrition plan is most appropriate to discuss with the patient and dietitian?

A) Two large high-fat meals with unrestricted fiber

B) Fasting during the day followed by one evening meal

C) Smaller, more frequent meals that are lower in fat and fiber, with glucose monitoring

D) Carbonated beverages with each meal to increase gastric emptying

36.

Scenario

A patient will take an oral hazardous antineoplastic medication at home. The caregiver says, “If swallowing gets hard, I will crush the tablet into applesauce with our kitchen pill crusher.”

Which teaching is appropriate?

A) Crush every dose in advance and store it in an open container

B) Ask pregnant household members to prepare the medication

C) Dispose of unused tablets in the household trash without packaging

D) Do not crush the tablet unless specifically directed; follow hazardous-drug handling and disposal instructions

37.

Scenario

An older adult with advanced dementia is nonverbal after hip surgery. The patient grimaces, guards the operative leg, and moans during repositioning, but vital signs are near baseline.

How should the nurse assess and manage possible pain?

A) Use a validated behavioral pain tool, treat likely pain, and reassess behavior and function

B) Withhold analgesia because the patient cannot provide a numeric rating

C) Use blood pressure as the only measure of pain intensity

D) Wait until the patient cries continuously before intervening

38.

Scenario

After lung surgery, a patient's chest drainage system suddenly develops continuous bubbling in the water-seal chamber. The patient is breathing comfortably, and all tubing remains below chest level.

Which action should the nurse take?

A) Clamp the chest tube near the patient and leave it clamped

B) Add sterile water to the collection chamber until bubbling stops

C) Inspect the system from the patient toward the drainage unit for a loose connection or air leak

D) Strip the tubing forcefully to clear an assumed clot

39.

Scenario

A patient with delirium repeatedly tries to get out of bed at night to urinate. The bed alarm sounds each time, but staff often arrive after the patient is already standing.

Which change is most likely to reduce fall risk?

A) Raise all four side rails and move the call bell out of reach

B) Administer a sedative at bedtime solely to prevent ambulation

C) Turn off the alarm because it has not prevented standing

D) Add purposeful rounding with scheduled toileting, a low bed, clear pathways, and needed mobility assistance

40.

Scenario

A patient with a large pressure injury eats less than one quarter of meals and has had unintentional weight loss. The patient says food tastes bland and fatigue makes finishing meals difficult.

Which nursing plan best supports wound healing?

A) Restrict protein until the wound drainage decreases

B) Request nutrition assessment and offer small, nutrient-dense, high-protein choices that match preferences

C) Replace meals with clear liquids until appetite returns

D) Delay intervention until a serum albumin result is available

PART B - ANSWER KEY AND EXPLANATIONS

Question numbering below corresponds to the scored questions in Part A.

1	B	2	D	3	A	4	C	5	B
6	A	7	D	8	A	9	D	10	C
11	B	12	B	13	A	14	C	15	D
16	B	17	C	18	C	19	D	20	B
21	A	22	A	23	A	24	D	25	B
26	C	27	B	28	A	29	D	30	D
31	A	32	C	33	A	34	B	35	C
36	D	37	A	38	C	39	D	40	B

1. Correct: B - Schedule assisted toileting, place a bedside commode within reach, and supervise transfers

Urgency, orthostatic symptoms, and impaired mobility create an immediate fall risk. Assisted toileting and a nearby commode reduce unsafe walking while maintaining mobility; bed rest and restraints add harm and do not address the causes.

2. Correct: D - An airborne infection isolation room with a fit-tested respirator, gown, and gloves

Suspected disseminated zoster requires airborne and contact precautions. A negative-pressure airborne infection isolation room plus respiratory, gown, and glove protection limits airborne and direct-contact transmission.

3. Correct: A - Hold the linezolid and promptly notify the prescriber of suspected serotonin toxicity

Agitation, diaphoresis, hyperreflexia, and clonus after combining linezolid with a serotonergic drug suggest serotonin toxicity. Further exposure should be stopped and the prescriber notified promptly for treatment and medication revision.

4. Correct: C - Stop opioid administration, support ventilation, and give naloxone per protocol

Marked sedation, bradypnea, and hypoxemia indicate opioid-induced respiratory depression. Airway and ventilation support plus timely reversal are required; simply applying oxygen without correcting hypoventilation is insufficient.

5. Correct: B - Practice paced breathing and guided imagery during pain flares

Relaxation-based strategies do not expose insensate skin to thermal injury. High heat, prolonged direct ice, and vigorous massage are unsafe where protective sensation is reduced.

6. Correct: A - Decline to witness and notify the proceduralist that valid consent must be obtained when the patient has decision-making capacity

The patient cannot demonstrate understanding after sedation, so the nurse should not attest to a signature obtained without current decision-making capacity. The proceduralist must resolve consent before a nonemergent procedure proceeds.

7. Correct: D - Keep the patient NPO and request a swallowing evaluation

Coughing and a wet voice after water suggest dysphagia and aspiration risk. Oral food, fluid, and medication should be withheld until swallowing safety is evaluated and an appropriate plan is established.

8. Correct: A - Delay the prandial insulin until the meal is available, following the medication order and facility timing policy

Rapid-acting prandial insulin given long before food could cause hypoglycemia. Coordinating administration with actual meal availability preserves intended glucose coverage and safety.

9. Correct: D - Activate an urgent response for possible neck hematoma and prepare to support the airway

Restlessness, repeated swallowing, neck swelling, and voice change after thyroid surgery suggest expanding hematoma with impending airway compromise. Urgent escalation and airway readiness take precedence over routine comfort measures.

10. Correct: C - Report the near miss through the safety reporting system and participate in review of contributing system factors

A near miss reveals a hazardous process even when no patient is harmed. Reporting supports system learning and prevention; blame-focused charting does not substitute for a safety report.

11. Correct: B - “Keep your vitamin K intake reasonably consistent and report major diet changes.”

Large changes in vitamin K intake can alter warfarin effect. Consistency is safer than unnecessary elimination of nutritious foods, with monitoring and communication when dietary patterns change.

12. Correct: B - Replace the dressing promptly using aseptic technique and the approved skin antiseptic

A damp or loose central-line dressing no longer provides a reliable barrier and should be changed promptly with aseptic technique. Reinforcing a compromised dressing delays needed infection prevention.

13. Correct: A - Assess the patient's sedation and breathing, then explain that only the patient may press the PCA button

PCA-by-proxy can deliver opioid when the patient is too sedated to request it, defeating a key safety feature. The nurse should assess for respiratory depression and stop the unsafe practice.

14. Correct: C - Lean slightly forward and use slow pursed-lip exhalation

Forward positioning and pursed-lip breathing can reduce air trapping, slow exhalation, and lessen dyspnea. Breath-holding, lying flat, and paper-bag breathing may worsen distress or gas exchange.

15. Correct: D - Match two approved patient identifiers against the order and blood product with the required independent verification

Two approved identifiers and the required blood-product verification reduce wrong-patient transfusion risk. Room number, diagnosis, or another person's statement are not acceptable identifiers.

16. Correct: B - Stop further sedation, open the airway, and call for immediate assistance

Snoring, reduced responsiveness, and hypoxemia indicate loss of airway patency and sedation deeper than intended. The nurse must halt sedative administration, support the airway, and summon help rather than allow the procedure to continue.

17. Correct: C - Clarify the clinical indication with the prescriber before starting treatment

Bacteriuria alone in an asymptomatic catheterized patient commonly represents colonization. Clarifying the indication helps avoid unnecessary antibiotic exposure and related harms while allowing the clinician to reassess for true infection.

18. Correct: C - Stop the infusion and manage the suspected infiltration according to the medication-specific protocol

Burning, swelling, and coolness are consistent with infiltration. The infusion should be stopped immediately; subsequent catheter handling, aspiration, compresses, and antidote decisions depend on the agent and protocol.

19. Correct: D - "We can combine strategies to reduce pain enough to improve sleep and function."

Chronic pain care generally targets meaningful improvement in comfort, activity, and quality of life rather than guaranteed elimination of all pain. Multimodal goals also reduce overreliance on opioids.

20. Correct: B - Stay with the patient and directly assess suicidal thoughts, plan, means, and immediate safety

The statement signals possible suicide risk and requires a direct, immediate assessment while maintaining safety. Asking clearly about suicide does not create suicidal intent and should not be deferred.

21. Correct: A - Elevate the head of the bed to at least 30 degrees during feeding

Head-of-bed elevation during gastric feeding reduces aspiration risk when not contraindicated. Blue dye and very frequent residual checks do not replace safe positioning.

22. Correct: A - Verify the regimen using medication containers, pharmacy records, and recent discharge information before finalizing the list

Medication reconciliation requires comparison of the best available sources when histories conflict. Choosing one list or relying only on memory can perpetuate omissions, duplications, or discontinued drugs.

23. Correct: A - Arrange a private, trauma-informed interview using a qualified interpreter if needed and follow the facility safety protocol

Private, nonjudgmental assessment separates the patient from a potentially controlling person and supports safe disclosure. Confrontation or using the companion as interpreter can increase danger.

24. Correct: D - "I will multiply sodium per serving by the number of servings I actually eat."

Nutrition labels list sodium per serving, so total intake depends on portions consumed. "Natural" labeling and avoiding only table salt do not ensure a low-sodium diet.

25. Correct: B - Keep the patient NPO and obtain urgent medical evaluation for possible perforation

Rapidly worsening pain, rigidity, distention, and tachycardia after colonoscopy are concerning for perforation. Oral intake, heat, and routine ambulation could delay urgent diagnosis and treatment.

26. Correct: C - Stop, assess for mechanical causes, and follow the facility's occlusion-management protocol

Flushing against resistance can damage the catheter or dislodge a thrombus. The nurse should check clamps, tubing position, and other mechanical causes, then use the approved occlusion pathway.

27. Correct: B - Use contact precautions and wash hands with soap and water after care

Norovirus spreads readily through contact with contaminated hands, surfaces, and body fluids. During outbreaks, contact precautions and soap-and-water handwashing after care are appropriate; shared equipment must be disinfected.

28. Correct: A - Failure mode and effects analysis

Failure mode and effects analysis prospectively identifies how a new process could fail and prioritizes preventive controls. Root cause analysis is generally performed after an event.

29. Correct: D - Stop the infusion, support circulation and airway as needed, and notify the anesthesia team immediately

New motor weakness, profound hypotension, and drowsiness during epidural therapy can signal excessive neuraxial effect or another serious complication. The infusion should be stopped and the patient supported and urgently evaluated.

30. Correct: D - Notify the prescriber and connect the patient with medication-assistance resources before discharge

Intermittent anticoagulant use can leave the patient unprotected and self-adjusted dosing can cause harm. Coordinating an affordable, prescribed plan addresses the cause of nonadherence safely.

31. Correct: A - Position the patient with knees flexed, cover the area with sterile saline-moistened dressings, and notify the surgeon urgently

Evisceration requires reduced abdominal tension, protection of exposed tissue from drying, and urgent surgical management. The organs must not be pushed back into the abdomen.

32. Correct: C - Assess causes of agitation and use the least restrictive effective alternatives with frequent reassessment

Restraints can worsen agitation and must be a last resort. Addressing pain, toileting, environment, communication, and tube concealment may permit safer, less restrictive care.

33. Correct: A - Place a surgical mask on the patient, limit transport to what is necessary, and notify the receiving department

A patient on airborne precautions should wear a surgical mask during necessary transport for source control. Advance notice allows the destination to maintain precautions and minimize exposure.

34. Correct: B - Help the patient splint the incision and coordinate deep breathing with the pain-management plan

Incision splinting plus well-timed analgesia makes deep breathing, coughing, and incentive spirometry more tolerable and effective. Avoidance and bed rest increase postoperative pulmonary risk.

35. Correct: C - Smaller, more frequent meals that are lower in fat and fiber, with glucose monitoring

Smaller meals and lower fat and fiber content can reduce delayed gastric emptying symptoms. Glucose monitoring is important because variable emptying can destabilize glycemic control.

36. Correct: D - Do not crush the tablet unless specifically directed; follow hazardous-drug handling and disposal instructions

Crushing can alter drug delivery and expose household members to hazardous particles. The dispensing pharmacy's specific handling, storage, administration, and disposal instructions should be followed.

37. Correct: A - Use a validated behavioral pain tool, treat likely pain, and reassess behavior and function

Inability to self-report does not exclude pain. A consistent behavioral tool, likely-pain treatment, and reassessment provide safer evidence than vital signs alone.

38. Correct: C - Inspect the system from the patient toward the drainage unit for a loose connection or air leak

New continuous water-seal bubbling suggests an air leak. Systematic inspection can locate a disconnection while avoiding prolonged clamping, which can trap air in the pleural space.

39. Correct: D - Add purposeful rounding with scheduled toileting, a low bed, clear pathways, and needed mobility assistance

This bundle addresses the patient's predictable toileting need and reduces injury and environmental hazards. Four side rails and sedatives can increase harm, while an alarm alone responds only after movement begins.

40. Correct: B - Request nutrition assessment and offer small, nutrient-dense, high-protein choices that match preferences

Weight loss, poor intake, and a large wound warrant early individualized nutrition support. Small preferred high-protein, energy-dense choices can improve intake; waiting for a single laboratory value delays care.